Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Full Study Title: A Phase Ib/II, Open-Label, Multicenter Study Evaluating the Safety, Activity, and Pharmacokinetics of Divarasib in Combination With Other Anti-Cancer Therapies in Participants With Previously Untreated Advanced Or Metastatic Non-Small Cell Lung Cancer With a KRAS G12C Mutation **Short Title/ Acronym:** KRAScendo 170 **Protocol Number:** BO44426 **NCT Number:** NCT05789082 **Posting ID:** 486683825977732271 **Sponsor/Company Name:** Hoffmann-La Roche (Roche) **Investigational Product:** Divarasib (in combination with Pembrolizumab, ± Carboplatin/Cisplatin [Trade Name: Platinol, ATC Code: L01XA01], and Pemetrexed) **Phase of Development:** Phase 1 / Phase 2 (Phase Ib/II) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, pharmacokinetics (PK), and clinical activity of divarasib combined with other anti-cancer therapies in participants with previously untreated, advanced or metastatic non-small cell lung cancer (NSCLC) harboring a KRAS G12C mutation. **Secondary Objectives:** To evaluate the objective response rate (ORR), progression-free survival (PFS), and duration of response (DOR) as assessed by the investigator per RECIST v1.1.

2.2 Background & Rationale The KRAS G12C mutation, present in approximately 12% of NSCLC patients, drives oncogenic signaling and cancer formation and is associated with poor prognosis. The current first-line treatment for advanced KRAS G12C+ NSCLC is a checkpoint inhibitor (CPI) ± chemotherapy (CT). Novel combinations using a more targeted, biomarker-directed approach are supported by pre-clinical evidence and may further improve outcomes. Divarasib is a potent oral KRAS G12C inhibitor. This study hypothesizes that divarasib + CPI ± CT may improve clinical outcomes for treatment-naive patients with KRAS G12C+ NSCLC.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Dose Finding and Dose Expansion stages) **Control Type:** Active Comparator / Parallel dose level randomisation in expansion stage **Blinding:** Open-label **Randomization:** Patients in the expansion stage will be randomized to different divarasib combination dose levels. **Trial Status:** Recruiting

3.2 Planned Enrollment & Duration Targeted Enrollment (N): 240 **Number of Sites:** Multicenter (Global) **Estimated Study Start:** Q1 2023 **Estimated Primary Completion:** Ongoing

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years with an Eastern Cooperative Oncology Group (ECOG) performance status (PS) of 0 or 1. 2. Histologically or cytologically documented locally advanced unresectable or metastatic non-squamous NSCLC that is not eligible for curative surgery and/or definitive chemoradiotherapy. 3. Confirmation of Biomarker eligibility: Confirmed KRAS G12C mutation via pre-treatment tumor tissue. Representative tumor specimens must be in formalin-fixed, paraffin embedded (FFPE) blocks (preferred) or 15 unstained, freshly cut, serial slides. If only 10 slides are available, the participant may be eligible following consultation with the Sponsor. 4. No prior systemic treatment for advanced unresectable or metastatic NSCLC. 5. Measurable disease, as defined by Response Evaluation Criteria in Solid Tumors (RECIST) v1.1.

4.2 Exclusion Criteria 1. Known concomitant second oncogenic driver with available targeted treatment. 2. Squamous cell histology NSCLC. 3. Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases. 4. Prior treatment with a KRAS G12C inhibitor. 5. Known hypersensitivity to any of the components of divarasib or pembrolizumab; or known hypersensitivity to pemetrexed, carboplatin, or cisplatin. 6. History of idiopathic pulmonary fibrosis, organizing pneumonia (e.g., bronchiolitis obliterans), drug-induced pneumonitis, or idiopathic pneumonitis, or evidence of active pneumonitis, active tuberculosis, or significant cardiovascular disease within 3 months prior to initiation of study treatment. 7. History of malignancy other than NSCLC within 5 years prior to initiation of study treatment, with the exception of malignancies with a negligible risk of metastasis or death (e.g., 5-year OS rate >90%), such as adequately treated carcinoma in situ of the cervix, non-melanoma skin carcinoma, localized prostate cancer, ductal breast carcinoma in situ, or Stage I uterine cancer. 8. Uncontrolled tumor-related pain, pleural effusion, pericardial effusion, or ascites requiring recurrent drainage procedures; uncontrolled or symptomatic hypercalcemia. 9. Co-morbid condition that is an absolute contraindication to treatment with corticosteroids. 10. Inability or unwillingness to take prophylactic treatments such as corticosteroids, anti-emetics, folic acid, or vitamin B12 supplementation.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Divarasib once daily (QD) on Days 1-21 of each 21-day cycle. Pembrolizumab (200 mg) is given every 3 weeks (Q3W) on Day 1 of each cycle. For certain cohorts, platinum-based chemotherapy (Carboplatin or Cisplatin) and pemetrexed are also administered. **Route of Administration:** Divarasib given orally (PO); Pembrolizumab and chemotherapy given via intravenous (IV) infusion. **Duration of Treatment:** Cycles are 21 days long. Participants will be treated until disease progression per RECIST v1.1, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Prophylactic treatments such as corticosteroids, anti-emetics, folic acid, and vitamin B12 supplementation. **Prohibited:** Other systemic anti-cancer treatments or unapproved investigational agents outside the study protocol.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Biomarker Confirmation, Baseline RECIST Tumor Assessment, Medical History | | **Cycle 1, Day 1** | Day 1 | Randomization (if applicable), First Doses (Divarasib PO + Pembrolizumab IV ± Chemo IV), PK Blood Draws | | **Treatment Period** | Every 21 Days | Cycle 1+ visits: Safety assessments, continued dosing. Tumor assessments every 6 weeks for 48 weeks, then every 9 weeks thereafter. | | **End of Study** | Variable | Safety follow-up, End of treatment tumor assessment, Survival Follow-up. |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of Participants with Adverse Events (AEs), including dose-limiting toxicities and changes from baseline in targeted safety parameters to determine the maximum tolerated dose and optimal combination dose. **Measurement Method:** Clinical examination, laboratory tests, and standard AE reporting criteria throughout the treatment period.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR) assessed per RECIST v1.1. 2. Duration of Response (DOR) assessed per RECIST v1.1. 3. Progression Free Survival (PFS) assessed per RECIST v1.1.

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring by the investigator, grading severity of toxicities. Co-primary focus of the Phase 1/2 evaluation. **Serious Adverse Events (SAEs):** Reported to the sponsor within 24 hours of investigator awareness. **Data Monitoring Committee (DMC):** Internal safety review committees will evaluate dose-limiting toxicities before proceeding to the dose expansion stage.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety-evaluable population for AE tracking; Intent-to-Treat (ITT) / Efficacy-evaluable population for measuring ORR, PFS, and DOR. **Sample Size Justification:** Designed to appropriately power the dose-finding and initial efficacy estimation stages for Phase Ib/II combination treatments (Target N=240). **Handling of Missing Data:** Censoring applied for survival and progression endpoints for participants lost to follow-up, standard for time-to-event oncology trials.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) guidelines and the Declaration of Helsinki. **Monitoring Plan:** Routine site visits and remote source data verification by clinical research associates to ensure protocol adherence. **Audit Trail:** Electronic Case Report Form (eCRF) utilizing 21 CFR Part 11 compliant platforms with automated data clarification mechanisms.

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11. Study Identifiers & Governance

Primary ID: BO44426 **Registry Number**: NCT05789082 **Posting ID**: 486683825977732271 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Study Title:** KRAScendo 170 **Official Regulatory Title:** A Phase Ib/II, Open-Label, Multicenter Study Evaluating the Safety, Activity, and Pharmacokinetics of Divarasib in Combination With Other Anti-Cancer Therapies in Patients With Previously Untreated Advanced Or Metastatic Non-Small Cell Lung Cancer With a KRAS G12C Mutation

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12. Clinical Framework (Oncology Specific)

Primary Condition / Disease Area: Non-Small Cell Lung Cancer (Preferred Name: Metastatic non-small cell lung cancer) **Preferred UMLS Name:** Non-Small Cell Lung Carcinoma, KRAS G12C Mutated **Semantic Type:** Neoplastic Process **MeSH Code:** D016938 **SNOMED ID**: 457721000124104 **Disease Definition:** Stage IV includes: (Any T, Any N, M1a); (Any T, Any N, M1b). M1a: Lung cancer with separate tumor nodule(s) in a contralateral lobe; tumor with pleural nodules or malignant pleural (or pericardial) effusion. M1b: Distant metastasis. (AJCC 7th ed.) **Medical Methodology:** Targeted therapy + Immunotherapy ± Platinum-based chemotherapy.

Optimization Target: Identifying the optimal dose and safety profile of divarasib combined with pembrolizumab ± chemotherapy.

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13. Study Procedures & Methodology

Management Pathway: Standard oncology treatment pathway integrated with experimental targeted KRAS G12C inhibitor. **Education Protocol (HCP):** Site initiation visits and detailed Investigator Brochure on managing targeted therapy and immunotherapy-related AEs. **Education Protocol (Patient):** Informed consent process detailing the dosing schedule and prophylactic regimen requirements (e.g., folic acid, vitamin B12). **Quality Audit Mechanism:** Centralized RECIST v1.1 review (if applicable) and regular safety monitoring checks.

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14. Observation & Follow-Up Schedule

Total Duration: Follow-up will continue until disease progression or death. **Onsite Visit Cadence:** Every 21 days (corresponding to treatment cycles). **Remote Monitoring Frequency:** Between cycles for safety/AE reporting as needed. **Checklist Review Interval:** Tumor assessments every 6 weeks for the first 48 weeks, then every 9 weeks.

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15. Sign-off & Approval

| Role | Name | Signature | Date | | :--- | :--- | :--- | :--- | | **Principal Investigator** | [Name] | ____ **| [DD-MMM-YYYY] | | Clinical Operations Lead | [Name] | ___ |** ***[DD-MMM-YYYY]*** **| |** ***Lead Statistician*** **|** ***[Name]*** **|** _____ | [DD-MMM-YYYY] |